



Before the Fall: Community-Based STEADI Screening to Identify Hidden Fall Risk Before ED Presentation

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WHY IT MATTERS

- Falls are the **leading cause of ED visits for trauma** among adults ≥ 65 .^{1,2}
- Multifactorial and multiple-component interventions **can reduce fall rates**.³
- STEADI — Stopping Elderly Accidents, Deaths, and Injuries — provides an evidence-based screen-assess-intervene framework.
- Primary care fall-prevention **screening remains inconsistent**.⁴

PILOT OBJECTIVES

- Identify** discordance between objective fall risk and self-perceived fall risk.
- Characterize** recent primary care fall-prevention engagement among screened participants.
- Assess** feasibility of community-based STEADI screening as a combined screening and education model.

PARTICIPANTS

4 events | Mean age 74.6 years | 73% female



THE AWARENESS GAP

1 in 2

high-risk older adults did **NOT** perceive their elevated fall risk

Community-based **STEADI** screening found **66%** of older adults at high fall risk — yet half **did not** recognize it, and most **had not** discussed fall prevention with their primary care provider

PILOT RESULTS AT A GLANCE



IMPLEMENTATION MODEL



WHAT WE FOUND

- 66%** met high-risk criteria
(all participants, $N=53$; STEADI ≥ 4)
- 50%** did not perceive elevated fall risk
(among high-risk participants)
- 45%** discussed fall prevention with a PCP
(all participants; prior year)
- 83%** had low concern about falling on FES-I
(among high-risk subgroup)

WHAT THIS MEANS

- Community-based STEADI screening identified older adults **with substantial, often unrecognized fall risk** outside routine clinical care.
- These findings support community screening as a **feasible upstream screening and intervention touchpoint** for larger-scale effectiveness testing.

LIMITATIONS

- A self-selected community health fair sample may underrepresent less ambulatory and/or higher-risk older adults.
- Follow-up completion was limited (**43.4%**), restricting longitudinal conclusions.

FUTURE DIRECTIONS

- Use **geospatial targeting** to expand STEADI screening to rural and high-burden Oregon communities with limited primary care access.
- Conduct a larger **Hybrid Type 1 Effectiveness-Implementation study** to test effects on fall-prevention engagement, intervention uptake, documented falls, and fall-related ED utilization.

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